Table 2: EMS NC II Competency-Based Multiple Choice Assessment (260 Items)

ID	Category	Question	Option A	Option B	Option C	Option D	Answer	Explanation
1	Core – Basic Life Support	What is the correct compression-to-ventilation ratio for adult CPR performed by a single rescuer?	15:2	30:2	30:1	50:2	B	The standard adult CPR ratio for a single rescuer is 30 compressions to 2 ventilations per AHA guidelines.
2	Core – Basic Life Support	When operating an AED, after the device analyzes and advises a shock, what should the rescuer do first?	Immediately press the shock button	Resume chest compressions	Ensure no one is touching the patient	Remove the AED pads	C	Before delivering a shock, the rescuer must ensure no one is in contact with the patient to prevent accidental shock to bystanders.
3	Core – Basic Life Support	What is the recommended depth of chest compressions for an adult victim?	1 to 2 inches	At least 2 inches (5 cm)	3 to 4 inches	0.5 inches	B	AHA guidelines recommend at least 2 inches (5 cm) compression depth for adults to ensure effective circulation.
4	Core – Basic Life Support	What is the correct compression rate for CPR?	60-80 compressions per minute	80-100 compressions per minute	100-120 compressions per minute	140-160 compressions per minute	C	The recommended compression rate is 100-120 compressions per minute for effective CPR.
5	Core – Basic Life Support	What is the compression-to-ventilation ratio for two-rescuer CPR on an adult?	15:2	30:2	30:1	50:2	B	For two-rescuer adult CPR, the ratio remains 30:2. The 15:2 ratio is used for pediatric two-rescuer CPR.
6	Core – Basic Life Support	What is the compression-to-ventilation ratio for two-rescuer CPR on a child?	30:2	15:2	30:1	5:1	B	For two-rescuer CPR on a child, the recommended ratio is 15 compressions to 2 ventilations.
7	Core – Basic Life Support	During CPR, what is the maximum allowable interruption to chest compressions?	30 seconds	20 seconds	10 seconds	60 seconds	C	Interruptions to chest compressions should be limited to no more than 10 seconds to maintain perfusion.
8	Core – Basic Life Support	After placing AED pads on a patient, what is the first action the AED performs?	Delivers a shock automatically	Analyzes the heart rhythm	Charges the capacitor	Records the ECG	B	Once pads are placed, the AED automatically analyzes the heart rhythm to determine if a shock is indicated.
9	Core – Basic Life Support	Which heart rhythm is most commonly associated with sudden cardiac arrest and is shockable?	Asystole	Ventricular fibrillation	Pulseless electrical activity	Sinus bradycardia	B	Ventricular fibrillation (VF) is the most common initial rhythm in sudden cardiac arrest and is shockable with defibrillation.
10	Core – Basic Life Support	What is the correct hand placement for chest compressions on an adult?	Upper third of the sternum	Lower half of the sternum	Over the xiphoid process	Left side of the chest over the apex	B	Compressions should be performed on the lower half of the sternum to effectively compress the heart between the sternum and spine.
11	Core – Basic Life Support	What should the rescuer allow during chest compressions to ensure proper chest recoil?	Full chest recoil between compressions	Partial chest recoil only	No recoil is needed	Recoil only every 5 compressions	A	Full chest recoil between compressions allows the heart to refill with blood, improving the effectiveness of CPR.
12	Core – Basic Life Support	For an infant, what technique is used for chest compressions?	Two hands	One hand	Two fingers (thumb and index)	Heel of one hand	C	For infant CPR, two fingers are placed on the lower sternum just below the nipple line to deliver compressions.
13	Core – Basic Life Support	When should the AED be used on a pediatric patient?	Only if the child is over 8 years old	When pediatric pads or a pediatric key is available	Never on pediatric patients	Only with a physician order	B	AEDs can be used on pediatric patients when pediatric pads or pediatric attenuation key are available to deliver a reduced energy shock.
14	Core – Basic Life Support	What is the recommended BLS assessment sequence for unresponsiveness?	Check pulse, then breathing	Check for responsiveness, then breathing and pulse simultaneously	Check breathing only	Check pupils first	B	The BLS sequence begins with checking responsiveness, followed by simultaneously assessing breathing and pulse (within 10 seconds).
15	Core – Basic Life Support	How long should the rescuer take to check for	At least 30 seconds	5-10 seconds	1-2 seconds	60 seconds	B	Assessment of breathing and pulse should take 5-10 seconds to minimize

		breathing and pulse in an unresponsive patient?						delays in starting CPR.
16	Core – Basic Life Support	What is the appropriate ventilation volume for an adult during CPR?	500-600 mL per breath	200-300 mL per breath	1000-1200 mL per breath	100 mL per breath	A	Each breath should be about 500-600 mL (enough to produce visible chest rise) over 1 second to avoid gastric inflation.
17	Core – Basic Life Support	What complication can result from excessive ventilation during CPR?	Improved oxygenation	Gastric inflation and regurgitation	Increased cardiac output	Better chest recoil	B	Excessive ventilation can cause gastric inflation, leading to regurgitation and aspiration, and also increases intrathoracic pressure reducing cardiac output.
18	Core – Basic Life Support	What does the 'CAB' sequence in BLS stand for?	Circulation, Airway, Breathing	Compress, Assess, Breathe	Check Airway, Breathe	Cardiac, Assessment, Breathing	A	CAB stands for Circulation, Airway, Breathing — the recommended sequence for BLS, emphasizing early chest compressions.
19	Core – Basic Life Support	If a patient is unresponsive but breathing normally with a pulse, what should the rescuer do?	Begin chest compressions	Place the patient in the recovery position and monitor	Apply the AED	Perform rescue breathing only	B	An unresponsive patient with normal breathing and a pulse should be placed in the recovery position to maintain the airway and monitored closely.
20	Core – Basic Life Support	After an AED delivers a shock, what is the immediate next step?	Check for a pulse	Resume chest compressions immediately	Remove the AED pads	Wait for the AED to re-analyze	B	After a shock is delivered, resume chest compressions immediately without checking for a pulse, to minimize interruptions.
21	Core – Basic Life Support	What is the correct jaw-thrust maneuver technique for opening the airway in a suspected spinal injury?	Tilt the head back and lift the chin	Place fingers behind the angle of the jaw and push upward	Push down on the forehead	Turn the head to the side	B	The jaw-thrust maneuver involves placing fingers behind the angles of the mandible and pushing upward, which opens the airway without extending the neck.
22	Core – Basic Life Support	What device is used to provide supplemental oxygen to a breathing patient during BLS?	Nasopharyngeal airway	Bag-valve-mask	Oropharyngeal airway	Chest decompression needle	B	A bag-valve-mask (BVM) is the primary device for providing positive-pressure ventilation with supplemental oxygen during BLS.
23	Core – Basic Life Support	When using a Bag-Valve-Mask (BVM), what is the recommended oxygen flow rate?	1-2 liters per minute	10-15 liters per minute	5-6 liters per minute	25-30 liters per minute	B	The BVM should be connected to supplemental oxygen at 10-15 L/min to deliver the highest possible oxygen concentration.
24	Core – Basic Life Support	Which of the following is a non-shockable rhythm?	Ventricular fibrillation	Ventricular tachycardia (pulseless)	Pulseless electrical activity	Both A and B	C	Pulseless electrical activity (PEA) is a non-shockable rhythm. VF and pulseless VT are shockable rhythms.
25	Core – Basic Life Support	What is the purpose of the head-tilt/chin-lift maneuver?	To check for injuries	To open the airway of an unresponsive patient without suspected spinal injury	To perform chest compressions	To deliver rescue breaths	B	The head-tilt/chin-lift maneuver opens the airway by displacing the tongue from the posterior pharynx in patients without suspected cervical spine injury.
26	Core – Life Support Equipment	What is the primary purpose of inspecting life support equipment before each shift?	To comply with administrative paperwork	To ensure all devices are functional and ready for use	To determine the purchase date of equipment	To assign equipment to specific personnel	B	Equipment inspection ensures that all life support devices are operational and ready for emergency use, directly affecting patient safety.
27	Core – Life Support Equipment	What should an EMS provider do if a portable suction unit fails during equipment check?	Ignore it and proceed with the shift	Document the failure and report for immediate repair or replacement	Wait until end of shift to report	Use it anyway and hope it works	B	Any equipment malfunction must be documented and reported immediately for repair or replacement to ensure availability during emergencies.
28	Core – Life Support Equipment	What is the standard oxygen cylinder color code used in many countries for medical oxygen?	Red	Green	White	Blue	C	Per ISO standards, medical oxygen cylinders are white-shouldered. In the US, green is used, but internationally white is the standard.
29	Core – Life Support Equipment	How should oxygen cylinders be stored?	Lying flat on the floor	Upright and secured to prevent falling	Near heat sources for quick access	In direct sunlight	B	Oxygen cylinders must be stored upright and secured to prevent falling and

								potential valve damage, which could cause a dangerous release of pressurized gas.
30	Core – Life Support Equipment	What is the function of a pulse oximeter?	Measures blood pressure	Measures oxygen saturation in the blood	Measures blood glucose levels	Measures carbon dioxide levels	B	A pulse oximeter non-invasively measures the percentage of hemoglobin saturated with oxygen (SpO2) in the blood.
31	Core – Life Support Equipment	What is the normal SpO2 reading for a healthy adult?	80-90%	94-100%	70-80%	50-60%	B	A normal SpO2 reading for a healthy adult is 94-100%. Readings below 90% indicate hypoxemia and require intervention.
32	Core – Life Support Equipment	What component of a Bag-Valve-Mask prevents rebreathing of exhaled air?	The reservoir bag	The one-way valve	The oxygen tubing	The pop-off valve	B	The one-way valve in the BVM directs exhaled air away from the rescuer and prevents rebreathing of exhaled gases.
33	Core – Life Support Equipment	What is the purpose of the pop-off valve on a BVM?	To increase ventilation pressure	To release excessive pressure and prevent barotrauma	To deliver more oxygen	To keep the mask sealed	B	The pop-off (pressure relief) valve vents excessive pressure to prevent barotrauma to the patient's lungs during positive-pressure ventilation.
34	Core – Life Support Equipment	Which suction catheter is most appropriate for suctioning the oropharynx of an adult?	6 Fr	8 Fr	14 Fr	4 Fr	C	A 14 Fr rigid suction catheter (Yankauer) is the standard for oropharyngeal suctioning in adults, providing adequate lumen size for thick secretions.
35	Core – Life Support Equipment	What should be checked on an oxygen regulator before use?	The color of the regulator only	The pressure gauge reading and integrity of connections	The manufacturing company name	The serial number only	B	The pressure gauge must show adequate cylinder pressure, and all connections must be intact and leak-free to ensure proper oxygen delivery.
36	Core – Life Support Equipment	What is the purpose of a non-rebreather mask?	To deliver low-flow oxygen	To deliver high-concentration oxygen (up to 90-95%)	To deliver medication	To measure oxygen levels	B	A non-rebreather mask with a reservoir bag can deliver up to 90-95% oxygen concentration at 10-15 L/min flow rate.
37	Core – Life Support Equipment	At what flow rate should a nasal cannula typically be set?	10-15 L/min	1-6 L/min	8-10 L/min	15-20 L/min	B	A nasal cannula is typically set at 1-6 L/min, delivering approximately 24-44% oxygen concentration.
38	Core – Life Support Equipment	What percentage of oxygen does a simple face mask deliver at 6-10 L/min?	24-28%	40-60%	80-90%	100%	B	A simple face mask at 6-10 L/min delivers approximately 40-60% oxygen concentration, as it mixes with room air.
39	Core – Life Support Equipment	What is the minimum number of AED pads that should be carried on an ambulance?	One set	Two sets (adult and pediatric)	Five sets	None required	B	Ambulances should carry at least two sets of AED pads — one adult and one pediatric — to accommodate all patient populations.
40	Core – Life Support Equipment	What is the purpose of a cervical collar in the EMS setting?	To treat neck pain	To immobilize the cervical spine and prevent further injury	To support the head during intubation	To keep the patient warm	B	A cervical collar immobilizes the cervical spine to prevent movement that could worsen a suspected spinal injury during assessment and transport.
41	Core – Life Support Equipment	What is the function of a Kendrick Extrication Device (KED)?	To transport a standing patient	To immobilize a seated patient during vehicle extrication	To deliver oxygen	To measure vital signs	B	The KED is a vest-type device designed to immobilize the torso, head, and neck of a seated patient during extrication from a vehicle.
42	Core – Life Support Equipment	How often should ambulance equipment be checked?	Weekly only	At the beginning of every shift	Monthly only	Only after each call	B	Equipment must be checked at the beginning of every shift to ensure all items are present, functional, and ready for immediate use.
43	Core – Life Support Equipment	What does the acronym 'PASS' stand for when using a fire extinguisher?	Pull, Aim, Squeeze, Sweep	Point, Activate, Stand, Shoot	Pull, Adjust, Squeeze, Save	Push, Aim, Squeeze, Sweep	A	PASS stands for Pull the pin, Aim at the base of the fire, Squeeze the handle, and Sweep from side to side.
44	Core – Life Support	What is the purpose of a cervical immobilization	To secure the	To prevent lateral movement of the	To elevate the	To measure cervical	B	A CID (head immobilizer) prevents lateral and rotational movement of the head and

	Equipment	device (CID) used with a long spine board?	patient's legs	head and neck	patient's head	range of motion		neck during spinal immobilization on a long spine board.
45	Core – Life Support Equipment	Which equipment documentation must be maintained by EMS providers?	Only purchase receipts	Equipment check logs, maintenance records, and usage logs	Only the manufacturer warranty	No documentation is required	B	EMS providers must maintain equipment check logs, maintenance records, and usage logs to ensure accountability and readiness of all life support equipment.
46	Core – Safe Access & Extrication	What is the first priority when arriving at a motor vehicle crash scene?	Immediately remove the patient from the vehicle	Establish scene safety	Administer oxygen to the patient	Contact the receiving hospital	B	Scene safety is always the first priority. Hazards must be assessed and managed before any rescue or patient contact.
47	Core – Safe Access & Extrication	Which device is most appropriate for cervical spine immobilization of a suspected spinal injury patient?	Soft cervical collar	Rigid cervical collar	Cervical traction device	Pelvic binder	B	A rigid cervical collar provides necessary immobilization to prevent further spinal cord injury during extrication, unlike a soft collar which only provides comfort.
48	Core – Safe Access & Extrication	What is the purpose of the rapid extrication technique?	To quickly move a patient whose condition or scene requires immediate removal	To save time on all calls	To avoid using equipment	To practice teamwork skills	A	Rapid extrication is used when the patient's condition or scene hazards require immediate removal, prioritizing speed over standard spinal precautions.
49	Core – Safe Access & Extrication	When should a patient be extricated using a long spine board?	Only for patients with obvious fractures	When spinal injury is suspected and standard extrication is indicated	For all patients regardless of injury	Only for unconscious patients	B	A long spine board is used for extrication when spinal injury is suspected, providing full-body immobilization during movement.
50	Core – Safe Access & Extrication	What is the correct technique for log-rolling a patient?	One rescuer pulls the patient	Multiple rescuers simultaneously rotate the patient as a unit while maintaining spinal alignment	The patient rolls themselves	Only the torso is moved	B	Log-rolling requires multiple rescuers to rotate the patient as a single unit, maintaining spinal alignment throughout the movement.
51	Core – Safe Access & Extrication	What is the minimum number of rescuers recommended for log-rolling a patient?	One	Two	Three	Five	C	At least three rescuers are needed: one at the head maintaining cervical alignment, and two along the body to control the roll.
52	Core – Safe Access & Extrication	What hazard should be assessed first at a vehicle crash scene?	Vehicle color	Traffic and oncoming vehicles	Patient's insurance information	Weather forecast	B	Traffic and oncoming vehicles pose an immediate life threat to rescuers and patients, and must be controlled before approaching the scene.
53	Core – Safe Access & Extrication	What does the term 'golden hour' refer to in trauma care?	The first 60 minutes from the time of injury to definitive care	The shift change period	The time of day with most accidents	The duration of ambulance transport	A	The golden hour refers to the critical first 60 minutes from injury to definitive surgical care, during which timely treatment significantly improves survival.
54	Core – Safe Access & Extrication	What is the correct sequence for securing a patient to a long spine board?	Legs, arms, torso, head	Torso first, then legs, then head	Head first, then torso, then legs	Arms, head, torso, legs	B	The torso is secured first to provide a stable base, then the legs, and finally the head with a cervical immobilization device.
55	Core – Safe Access & Extrication	What type of vehicle hazard requires immediate evacuation of the area?	A flat tire	Leaking fuel with risk of fire	A broken windshield	An open door	B	Leaking fuel with a risk of fire is a life-threatening hazard requiring immediate evacuation of the area until fire personnel declare it safe.
56	Core – Safe Access & Extrication	What is the purpose of stabilizing a vehicle before extrication?	To make the vehicle look presentable	To prevent the vehicle from moving and causing further injury to the patient or rescuers	To access the trunk	To turn off the engine	B	Vehicle stabilization prevents unexpected movement that could injure rescuers, worsen the patient's condition, or complicate extrication.
57	Core – Safe Access & Extrication	When using a stair chair, the patient should be positioned how?	Facing backward	Facing forward in a seated position	Lying flat	Standing upright	B	A stair chair is designed for a seated patient facing forward, allowing controlled descent on stairs while maintaining patient comfort and safety.

58	Core – Safe Access & Extrication	What is the primary concern when extricating a patient from water?	Getting the patient dry quickly	Maintaining spinal precautions while removing the patient from the water	Finding the patient's belongings	Swimming speed	B	When extricating from water, spinal precautions must be maintained using a backboard, as aquatic injuries frequently involve the cervical spine.
59	Core – Safe Access & Extrication	What is the recommended method for moving a patient with a suspected pelvic fracture?	Log-roll the patient	Use a scoop stretcher	Have the patient walk to the ambulance	Drag the patient by the arms	B	A scoop stretcher minimizes movement of the pelvis during transfer, reducing the risk of worsening a pelvic fracture and internal bleeding.
60	Core – Safe Access & Extrication	What PPE should rescuers wear during vehicle extrication?	Only gloves	Full PPE including helmet, eye protection, gloves, and reflective vest	Casual clothing	Only a reflective vest	B	Full PPE including helmet, eye protection, gloves, and reflective vest protects rescuers from flying debris, sharp metal, glass, and traffic hazards during extrication.
61	Core – Safe Access & Extrication	What is the correct position for the rescuer at the head during cervical spine immobilization?	Standing beside the patient	Kneeling behind the patient's head, holding it in neutral alignment	Sitting on the patient's chest	Holding the patient's shoulders	B	The rescuer kneels behind the patient's head, maintaining manual in-line stabilization in neutral alignment until a cervical collar and immobilization device are applied.
62	Core – Safe Access & Extrication	When is it acceptable to remove a motorcycle helmet from a patient?	Never	When it interferes with airway management or assessment	Always, immediately upon arrival	Only if the patient requests it	B	A helmet should be removed when it interferes with airway management or assessment, using a two-rescuer technique to maintain spinal alignment.
63	Core – Safe Access & Extrication	What is the recommended technique for removing a motorcycle helmet?	One rescuer pulls it off forcefully	Two-rescuer technique: one stabilizes the neck while the other removes the helmet	Cut the helmet off	Leave it on permanently	B	A two-rescuer technique ensures spinal alignment is maintained: one rescuer holds the head and neck while the other carefully removes the helmet.
64	Core – Safe Access & Extrication	What is a basket stretcher (Stokes litter) primarily used for?	Hospital transport	Moving patients over rough terrain or during technical rescue	Wheelchair transport	Stretching exercises	B	A basket stretcher is designed for moving patients over rough terrain, during technical rescue operations, or in confined spaces where standard stretchers cannot go.
65	Core – Safe Access & Extrication	What is the first step in the extrication process?	Cutting the roof of the vehicle	Assessing the scene and ensuring safety	Removing the patient immediately	Starting an IV line	B	Scene assessment and safety must be established before any extrication activities, identifying hazards and ensuring the area is safe for rescuers and the patient.
66	Core – Safe Access & Extrication	Which type of glass should be managed before patient extrication from a vehicle?	Only the windshield	All broken or unstable glass that could injure the patient or rescuers	No glass management is needed	Only the rear window	B	All broken or unstable glass must be managed (covered, removed, or stabilized) to prevent injury to the patient and rescuers during extrication operations.
67	Core – Safe Access & Extrication	What is the function of a spring-loaded center punch for vehicle extrication?	To measure tire pressure	To break vehicle windows for access	To open the hood	To check the battery	B	A spring-loaded center punch is used to break tempered glass windows for vehicle access, allowing rescuers to reach the patient quickly.
68	Core – Safe Access & Extrication	What is the correct body mechanics principle when lifting a patient?	Bend at the waist and lift with the back	Keep the back straight, bend at the knees, and lift with the legs	Twist the torso while lifting	Lift with arms extended	B	Proper body mechanics require keeping the back straight, bending at the knees, and lifting with the leg muscles to prevent back injuries.
69	Core – Safe Access & Extrication	What is the purpose of the 'power grip' when carrying a stretcher?	To look professional	To maximize grip strength and reduce the risk of dropping the stretcher	To carry with one hand	To reduce weight	B	The power grip involves gripping the stretcher handle with all fingers and the palm, maximizing strength and reducing the risk of dropping the patient.
70	Core – Safe Access &	How many rescuers are minimally required to	One	Two	Four	Six	B	At minimum, two rescuers are required to

	Extrication	safely carry a loaded stretcher?						carry a loaded stretcher — one at the head and one at the foot — though four is preferred for safety.
71	Core – Ambulance Service Management	In ambulance dispatch protocols, what does 'resource allocation' refer to?	Distributing medical supplies within the ambulance	Assigning the appropriate ambulance and crew to an emergency call based on priority and location	Allocating budget for ambulance maintenance	Dividing patient care tasks among EMS students	B	Resource allocation involves assigning the nearest and most appropriate ambulance unit and crew to respond to emergency calls based on priority and proximity.
72	Core – Ambulance Service Management	What is the primary purpose of a dispatch protocol in EMS?	To reduce the number of ambulance units	To ensure timely and appropriate response to emergency calls	To eliminate the need for paramedics	To track fuel consumption	B	Dispatch protocols standardize the process of receiving, prioritizing, and dispatching appropriate resources to emergency calls for efficient and effective response.
73	Core – Ambulance Service Management	What does the acronym 'START' stand for in triage?	Simple Triage And Rapid Treatment	Systematic Tracking And Response Team	Standard Treatment And Rescue Technique	Speed, Triage, Assessment, Rescue, Transport	A	START stands for Simple Triage And Rapid Treatment, a widely used mass casualty triage system that categorizes patients by severity.
74	Core – Ambulance Service Management	What is the purpose of mutual aid agreements between EMS agencies?	To compete with each other	To provide assistance when one agency's resources are overwhelmed	To share employee salaries	To reduce the number of ambulances	B	Mutual aid agreements allow agencies to share resources and personnel when one agency's capacity is exceeded during large-scale emergencies.
75	Core – Ambulance Service Management	What is the meaning of 'response time' in EMS?	The time taken to complete paperwork	The interval from dispatch notification to arrival at the scene	The time spent on scene	The duration of hospital transfer	B	Response time is measured from the moment the unit is dispatched to the moment it arrives at the scene, a key performance indicator for EMS systems.
76	Core – Ambulance Service Management	What information should be obtained during emergency call intake?	Only the caller's name	Location, nature of emergency, number of patients, and callback number	Only the address	The weather conditions only	B	Call intake must capture location, nature of emergency, number of patients, and a callback number to ensure appropriate resources are dispatched and verified.
77	Core – Ambulance Service Management	What is the role of the EMS dispatcher?	To treat patients at the scene	To receive emergency calls, determine priority, and dispatch appropriate units	To drive the ambulance	To perform surgery	B	The EMS dispatcher receives calls, determines the nature and priority of the emergency, and dispatches the appropriate units to the scene.
78	Core – Ambulance Service Management	What is a 'standing order' in EMS?	A permanent medication supply order	A preauthorized medical intervention that can be performed without direct physician contact	An order to remain on standby	A shift schedule order	B	Standing orders are preauthorized protocols that allow EMS providers to perform specific interventions without requiring real-time physician authorization.
79	Core – Ambulance Service Management	What is the purpose of an after-action review (AAR) following an EMS incident?	To assign blame to individuals	To evaluate performance and identify areas for improvement	To celebrate successes only	To complete paperwork requirements	B	An AAR is a structured review process to evaluate what happened, why it happened, and how to improve performance in future incidents.
80	Core – Ambulance Service Management	What is the difference between BLS and ALS units?	BLS units are faster than ALS units	BLS provides basic life support; ALS provides advanced life support with more interventions	ALS units cannot transport patients	There is no difference	B	BLS units provide basic interventions (CPR, AED, oxygen), while ALS units have paramedics who can perform advanced interventions (intubation, IV medications, cardiac monitoring).
81	Core – Ambulance Service Management	What is the role of the incident commander at an MCI?	To treat the most critically injured	To oversee all operations and coordinate resources at the incident scene	To transport patients	To communicate with the media only	B	The incident commander has overall responsibility for managing the incident, coordinating resources, and ensuring the safety of all personnel and patients.
82	Core – Ambulance Service Management	What does the term 'staging' mean in EMS operations?	Preparing for a theater performance	Holding units at a designated location until they are	Parking at the hospital	Returning to the station	B	Staging refers to holding ambulances and personnel at a designated area near the incident until they are given a specific

				assigned to a specific task				assignment by the incident commander.
83	Core – Ambulance Service Management	What is the function of a treatment sector in an MCI?	To triage patients	To provide medical assessment and treatment to sorted patients	To transport patients only	To communicate with the hospital	B	The treatment sector provides medical assessment and interventions to patients who have already been triaged and sorted by priority category.
84	Core – Ambulance Service Management	What is the recommended staffing minimum for a BLS ambulance unit?	One EMT	Two EMTs	Three paramedics	One physician	B	A BLS ambulance requires at minimum two EMTs to safely and effectively provide patient care and operate the ambulance.
85	Core – Ambulance Service Management	What is the meaning of 'turnaround time' in EMS?	The time to turn the ambulance around on the road	The time from arrival at the hospital until the unit is available for another call	The time spent driving to the scene	The shift duration	B	Turnaround time is the interval from arrival at the hospital (with patient) until the unit is cleaned, restocked, and available for the next call.
86	Core – Ambulance Service Management	What is a 'mass casualty incident' (MCI)?	An incident with one critical patient	An event where the number of patients exceeds available resources	A routine ambulance call	A hospital code blue	B	An MCI is any event where the number and severity of patients exceeds the available resources of the responding EMS system.
87	Core – Ambulance Service Management	What priority level is typically assigned to a patient with a life-threatening condition in dispatch?	Priority 3 (non-urgent)	Priority 1 (emergent/life-threatening)	Priority 4 (scheduled)	No priority assigned	B	Priority 1 indicates a life-threatening emergency requiring immediate response with lights and siren.
88	Core – Ambulance Service Management	What is the purpose of a pre-arrival instruction given by a dispatcher?	To keep the caller occupied	To provide the caller with medical instructions before EMS arrival	To collect payment information	To determine the caller's location only	B	Pre-arrival instructions (PAI) guide callers through life-saving interventions such as CPR or bleeding control before EMS personnel arrive on scene.
89	Core – Ambulance Service Management	What document outlines the scope of practice for EMS providers?	The ambulance owner's manual	The TESDA Training Regulations and local EMS laws	The hospital admission form	The driver's license	B	The scope of practice is defined by the TESDA Training Regulations, national laws, and local regulatory bodies that govern what interventions each level of EMS provider may perform.
90	Core – Ambulance Service Management	What is the purpose of quality improvement (QI) in EMS?	To punish providers for errors	To continuously evaluate and improve the quality of patient care and system performance	To increase the number of calls	To reduce staff numbers	B	QI programs systematically evaluate care delivery, identify deficiencies, and implement improvements to enhance patient outcomes and system efficiency.
91	Core – Ambulance Communication	When communicating via radio with a hospital, which technique should be used?	Speak as quickly as possible	Use clear, concise, and standardized medical terminology	Use slang and jargon for speed	Speak in the local dialect only	B	Radio communication must use clear, concise, and standardized medical terminology to ensure accurate and unambiguous information transfer.
92	Core – Ambulance Communication	What is the standard format for giving a radio report to a receiving hospital?	Random order of information	Unit identification, patient assessment, treatment given, and ETA	Only the patient's name	Only the destination	B	The standard format includes unit identification, patient assessment findings, interventions performed, and estimated time of arrival.
93	Core – Ambulance Communication	What does the acronym 'SBAR' stand for in medical communication?	Situation, Background, Assessment, Recommendation	Symptoms, Breathing, Airway, Rescue	Standard, Basic, Assessment, Response	Systolic, Blood, Assessment, Rate	A	SBAR is a structured communication framework: Situation, Background, Assessment, Recommendation — used for clear and concise clinical handoffs.
94	Core – Ambulance Communication	What is the primary reason for using standardized radio codes in EMS?	To make communication more difficult	To ensure clear and consistent communication across agencies	To keep information secret from the public	To reduce the need for training	B	Standardized codes ensure consistent, unambiguous communication between EMS providers, dispatchers, and receiving facilities across different agencies.
95	Core – Ambulance Communication	When communicating with a confused or agitated patient, what technique is most effective?	Speak loudly and authoritatively	Use a calm, reassuring tone and simple language	Ignore the patient	Use medical jargon to sound professional	B	A calm, reassuring tone with simple language helps reduce patient anxiety and improves cooperation during assessment and treatment.

96	Core – Ambulance Communication	What information must be included in the patient care report (PCR)?	Only the patient's name and address	Patient assessment, treatment provided, changes in condition, and all communications	Only the vital signs	Only the transport destination	B	A PCR must comprehensively document patient assessment findings, interventions, patient response, and all relevant communications to serve as a legal and medical record.
97	Core – Ambulance Communication	What is the purpose of a verbal handoff report when transferring patient care?	To delay patient care	To communicate critical patient information clearly and efficiently to the receiving provider	To avoid writing a PCR	To socialize with hospital staff	B	A verbal handoff communicates essential patient information — condition, treatment, and response — to the receiving provider for continuity of care.
98	Core – Ambulance Communication	Which of the following is an example of a closed-loop communication technique?	Giving an order without confirmation	Repeating back the order to confirm understanding before execution	Ignoring the instruction	Writing notes without verbal exchange	B	Closed-loop communication involves the receiver repeating back the order, the sender confirming, ensuring the message was accurately received and understood.
99	Core – Ambulance Communication	What is the HIPAA regulation regarding patient information communicated by radio?	No restrictions apply	Only the minimum necessary information should be transmitted	All patient details must be broadcast	HIPAA does not apply to EMS	B	HIPAA requires that only the minimum necessary patient information be transmitted over radio to protect patient privacy while ensuring continuity of care.
100	Core – Ambulance Communication	When should an EMS provider use the term 'copy' during radio communication?	To request a document	To confirm that a message has been received and understood	To end the radio transmission permanently	To request a repeat of the message	B	'Copy' is used to confirm that the transmitted message has been received and understood by the listener.
101	Core – Ambulance Communication	What is the proper procedure if radio communication is interrupted during a report?	Continue speaking without stopping	Stop, identify yourself, and repeat the last transmitted information	Hang up and call again later	Switch to a personal cell phone	B	If interrupted, the provider should stop, re-identify, and repeat the last transmitted portion to ensure complete and accurate information transfer.
102	Core – Ambulance Communication	What is the '10-code' system used for in some EMS communication systems?	Medical diagnosis	Brief standardized radio codes for common messages	Patient billing codes	Equipment inventory codes	B	10-codes are standardized numeric codes used on radio for brevity (e.g., 10-4 = acknowledgment), though many agencies now prefer plain language.
103	Core – Ambulance Communication	Why is plain language preferred over 10-codes in modern EMS communication?	It is shorter	It reduces miscommunication during multi-agency responses	It sounds less professional	10-codes are no longer taught	B	Plain language is preferred because 10-codes vary between agencies, causing confusion during multi-agency or mutual aid responses; plain language is universally understood.
104	Core – Ambulance Communication	What is the purpose of documenting all radio communications in the PCR?	To fill space in the report	To create a legal record of all orders, reports, and communications for accountability and continuity	To test the radio equipment	To count the number of radio calls	B	Documenting all radio communications creates a legal record that demonstrates accountability, supports continuity of care, and protects the provider.
105	Core – Ambulance Communication	When giving a patient report, what should be communicated about the patient's mental status?	Only whether the patient is awake or asleep	Level of responsiveness using AVPU or GCS scale	The patient's mood only	Whether the patient is cooperative	B	Mental status should be reported using a standardized scale (AVPU: Alert, Verbal, Pain, Unresponsive or GCS) for objective, consistent communication.
106	Core – Ambulance Communication	What does AVPU stand for in patient communication?	Airway, Ventilation, Pulse, Unconscious	Alert, Verbal, Pain, Unresponsive	Assessment, Vital signs, Pain, Urgency	Airway, Voice, Pulse, Understanding	B	AVPU is a simplified mental status assessment scale: Alert, responds to Verbal stimuli, responds to Pain, or Unresponsive.
107	Core – Ambulance Communication	How should an EMS provider communicate with a pediatric patient?	Use the same approach as with adults	Use age-appropriate language, a gentle tone, and involve the parent or guardian	Ignore the child and only talk to the parent	Use medical jargon to appear knowledgeable	B	Pediatric communication requires age-appropriate language, a gentle and reassuring tone, and involvement of parents or guardians to gain the child's trust and cooperation.
108	Core – Ambulance	What is the purpose of a 'patch' call in EMS radio	To repair the radio	To connect with a	To change radio	To test the antenna	B	A patch call connects the EMS provider

	Communication	communication?		medical direction physician for orders or consultation	channels			with medical direction (online medical control) to receive treatment orders or consultation for patient care decisions.
109	Core – Ambulance Communication	What is 'off-line medical direction' in EMS?	Physician is not available	Standing orders and protocols pre-approved by medical direction	No medical oversight	Communication by letter only	B	Off-line (indirect) medical direction refers to pre-approved protocols and standing orders written by the medical director that guide care without real-time physician contact.
110	Core – Ambulance Communication	What should an EMS provider do if they disagree with a medical direction order?	Ignore the order completely	Clarify the order respectfully, state concerns, and document the exchange	Follow the order without question	File a lawsuit immediately	B	Providers should respectfully clarify the order and express concerns. If still directed to proceed, they should document the exchange including the order and their concerns.
111	Core – Emergency Scene Management	In a mass casualty incident, which triage category is assigned to patients with life-threatening but treatable injuries?	Green (Minor)	Yellow (Delayed)	Red (Immediate)	Black (Deceased/Expectant)	C	Red-tagged patients have life-threatening but treatable injuries requiring immediate intervention — they are the highest treatment priority.
112	Core – Emergency Scene Management	What is the primary purpose of hazard assessment at an emergency scene?	To determine the cost of damages	To identify potential dangers to rescuers and patients before providing care	To write a report for the local government	To identify witnesses	B	Hazard assessment identifies risks such as chemical spills, fire, or structural collapse to protect rescuers and patients before care is initiated.
113	Core – Emergency Scene Management	What does the 'I' in the Incident Command System (ICS) represent?	Infection	Incident	Intervention	Inspection	B	ICS stands for Incident Command System — a standardized management structure for coordinating emergency response operations.
114	Core – Emergency Scene Management	What is the first step in the START triage process?	Treat all injuries	Assess all patients for the ability to walk (walking wounded)	Start IV lines	Transport the most injured first	B	The first step in START triage is to identify walking wounded patients who are tagged green (minor), allowing focus on non-ambulatory patients.
115	Core – Emergency Scene Management	In START triage, what is the respiratory rate threshold for immediate (red) tagging?	Less than 10 breaths per minute	Greater than 30 breaths per minute	12-20 breaths per minute	Exactly 16 breaths per minute	B	In START triage, a respiratory rate greater than 30 per minute indicates a red (immediate) tag, signifying respiratory distress requiring urgent intervention.
116	Core – Emergency Scene Management	What color tag is assigned to patients with minor injuries in the START triage system?	Red	Yellow	Green	Black	C	Green tags are assigned to walking wounded patients with minor injuries who can wait for treatment without significant risk of deterioration.
117	Core – Emergency Scene Management	What does a black triage tag indicate?	Patient needs immediate surgery	Patient is deceased or has injuries incompatible with survival	Patient has minor injuries	Patient is the first to be treated	B	A black tag indicates the patient is deceased or has fatal injuries incompatible with survival given available resources in an MCI.
118	Core – Emergency Scene Management	What is the purpose of establishing a command post at an emergency scene?	To provide refreshments	To centralize decision-making and coordination of all scene operations	To park ambulances	To treat patients	B	A command post centralizes incident management, providing a single point for decision-making, resource coordination, and communication.
119	Core – Emergency Scene Management	What is the role of the triage officer at an MCI?	To treat patients	To rapidly assess and categorize all patients by severity	To drive ambulances	To communicate with the media	B	The triage officer rapidly assesses each patient and assigns a priority category, ensuring the most critical patients are identified and treated first.
120	Core – Emergency Scene Management	What is scene control in EMS?	Controlling the ambulance speed	Managing access to the scene and maintaining order for safe and effective operations	Controlling the patient's behavior	Controlling the radio frequency	B	Scene control involves managing who enters and exits the incident area, maintaining safety zones, and ensuring orderly operations for effective patient care.
121	Core – Emergency	What is the 'warm zone' in a hazardous materials	The area completely	The area where	The area of maximum	The hospital	B	The warm zone is the contamination

	Scene Management	incident?	safe from all hazards	contamination reduction occurs, between hot and cold zones	contamination	emergency department		reduction corridor between the hot zone (contaminated area) and cold zone (safe area), where decontamination occurs.
122	Core – Emergency Scene Management	What is the 'cold zone' in a hazardous materials incident?	The area of maximum contamination	The safe area where command post and support functions are located	The decontamination area	The area inside the hazard	B	The cold zone is the safe area free from contamination where the command post, treatment, and support operations are established.
123	Core – Emergency Scene Management	What is the 'hot zone' in a hazardous materials incident?	The area with warm weather	The area of maximum contamination and danger requiring full PPE	The hospital zone	The patient waiting area	B	The hot zone is the contaminated area with the highest danger level, requiring specialized PPE and trained personnel for entry.
124	Core – Emergency Scene Management	When should an EMS provider enter a hazardous materials hot zone?	Whenever they feel it is necessary	Only with appropriate PPE, training, and under the direction of the incident commander	Only during daylight hours	Never under any circumstances	B	Entry into the hot zone requires appropriate PPE, hazmat training, and authorization from the incident commander to ensure rescuer safety.
125	Core – Emergency Scene Management	What is the primary goal of scene size-up?	To determine patient identity	To identify hazards, estimate resource needs, and determine the nature of the incident	To start treatment immediately	To locate valuables	B	Scene size-up rapidly identifies hazards, the nature and scope of the incident, and the resources needed to mount an effective response.
126	Core – Emergency Scene Management	What principle guides the order of patient treatment in an MCI?	First come, first served	Treat the greatest number with the greatest chance of survival	Treat the most severely injured first regardless of survivability	Treat children first only	B	MCI triage prioritizes treating the greatest number of patients who have the greatest chance of survival, maximizing overall positive outcomes.
127	Core – Emergency Scene Management	What is the function of a staging area during an MCI?	To treat patients	To hold incoming resources until they are assigned to a specific task	To decontaminate patients	To discharge patients	B	The staging area holds arriving units and resources in an organized manner until the incident commander assigns them to specific tasks or sectors.
128	Core – Emergency Scene Management	What is the role of the transportation officer in an MCI?	To perform surgery	To coordinate patient distribution to appropriate hospitals	To drive the ambulance only	To repair vehicles	B	The transportation officer coordinates the distribution of patients to receiving hospitals, ensuring no single facility is overwhelmed and patients go to appropriate levels of care.
129	Core – Emergency Scene Management	In the JumpSTART pediatric triage system, what is the first assessment step?	Check blood pressure	Assess if the child can walk	Start an IV	Check pupil response	B	Similar to START, JumpSTART begins by identifying children who can walk (green tag), allowing focus on non-ambulatory pediatric patients.
130	Core – Emergency Scene Management	What is the purpose of an incident action plan (IAP)?	To document the incident for media	To outline the objectives, strategies, and tactics for managing the incident	To order food for rescuers	To assign parking spots	B	An IAP documents the incident objectives, strategies, and tactical actions to ensure all responders work toward common goals in a coordinated manner.
131	Core – Emergency Scene Management	What is the minimum PPE required for EMS personnel at a crash scene?	No PPE is required	Gloves, eye protection, and reflective vest at minimum	Full hazmat suit	Only a helmet	B	Minimum PPE at a crash scene includes gloves for body substance isolation, eye protection, and a reflective vest for visibility in traffic.
132	Core – Emergency Scene Management	What should an EMS provider do if they discover a secondary device or hazard at a scene?	Attempt to disarm or remove it	Immediately evacuate and notify the incident commander	Ignore it and continue patient care	Take a photograph for social media	B	If a secondary hazard is discovered, providers must immediately evacuate the area and notify the incident commander for specialist assessment.
133	Core – Emergency Scene Management	What does 'size-up' involve in emergency scene management?	Only counting the number of vehicles	Rapid visual assessment of the scene for hazards, patient count, and	Measuring the scene dimensions	Interviewing all bystanders	B	Size-up is a rapid visual assessment that identifies hazards, estimates the number of patients, and determines the resources needed for the response.

				resource needs				
134	Core – Emergency Scene Management	What is the role of law enforcement at an EMS incident scene?	To provide medical treatment	To ensure scene safety, traffic control, and crowd management	To drive the ambulance	To perform triage	B	Law enforcement provides scene safety, traffic control, crowd management, and investigation support, allowing EMS to focus on patient care.
135	Core – Emergency Scene Management	What is the principle of 'doing the most good for the most people' associated with?	Routine patient care	Mass casualty incident triage	Hospital administration	Ambulance maintenance	B	This principle guides MCI triage decisions, prioritizing care for patients with the best chance of survival to maximize overall positive outcomes.
136	Core – Pre-Hospital Patient Care	What is the normal resting heart rate range for a healthy adult?	40-60 bpm	60-100 bpm	100-120 bpm	120-140 bpm	B	The normal resting heart rate for a healthy adult is 60-100 bpm. Values outside this range may indicate bradycardia or tachycardia.
137	Core – Pre-Hospital Patient Care	What does the mnemonic 'SAMPLE' stand for?	Signs/Symptoms, Allergies, Medications, Past medical history, Last oral intake, Events leading up	Size, Age, Medical history, Pain level, Level of consciousness, Emergency contact	Scene safety, Assessment, Management, Priorities, Logistics, Evacuation	Systolic, Airway, Movement, Pulse, Lungs, Eyes	A	SAMPLE is a medical history mnemonic: Signs/Symptoms, Allergies, Medications, Past medical history, Last oral intake, Events leading up to illness/injury.
138	Core – Pre-Hospital Patient Care	What is the normal blood pressure range for a healthy adult?	80/40 to 90/50 mmHg	90/60 to 120/80 mmHg	140/90 to 160/100 mmHg	160/100 to 180/120 mmHg	B	Normal adult blood pressure is approximately 90/60 to 120/80 mmHg. Readings above 120/80 may indicate prehypertension or hypertension.
139	Core – Pre-Hospital Patient Care	What is the normal respiratory rate for a healthy adult?	6-10 breaths per minute	12-20 breaths per minute	25-35 breaths per minute	35-45 breaths per minute	B	The normal respiratory rate for a healthy adult is 12-20 breaths per minute. Rates outside this range may indicate respiratory distress or failure.
140	Core – Pre-Hospital Patient Care	What does the 'P' in the OPQRST mnemonic stand for?	Position	Provocation	Pulse	Pressure	B	In OPQRST (pain assessment), P stands for Provocation — what provokes or palliates the pain, asking what makes it better or worse.
141	Core – Pre-Hospital Patient Care	What does the 'Q' in OPQRST stand for?	Quick	Quality	Quantity	Quadrant	B	Q stands for Quality — asking the patient to describe the character of the pain (sharp, dull, burning, crushing, etc.).
142	Core – Pre-Hospital Patient Care	What does the 'R' in OPQRST stand for?	Respiration	Radiation	Rate	Reaction	B	R stands for Radiation — asking whether the pain radiates or moves to other areas of the body.
143	Core – Pre-Hospital Patient Care	What does the 'S' in OPQRST stand for?	Signs	Severity	Symptoms	Skin	B	S stands for Severity — asking the patient to rate the pain on a scale of 0-10 to quantify the intensity.
144	Core – Pre-Hospital Patient Care	What does the 'T' in OPQRST stand for?	Treatment	Time	Temperature	Tenderness	B	T stands for Time — asking when the pain or symptom started and its duration, which helps determine the onset and progression.
145	Core – Pre-Hospital Patient Care	What is the first step in patient assessment?	Check vital signs	Scene size-up and ensure scene safety	Administer medications	Start an IV	B	Patient assessment begins with scene size-up and ensuring scene safety before approaching the patient, protecting both the provider and the patient.
146	Core – Pre-Hospital Patient Care	What is the correct technique for measuring blood pressure?	Inflate the cuff to 200 mmHg regardless	Inflate 30 mmHg above the point where the radial pulse disappears, then deflate slowly	Inflate to 100 mmHg only	Do not use a stethoscope	B	Proper technique involves inflating the cuff 30 mmHg above the palpated radial pulse disappearance point, then deflating slowly at 2-3 mmHg per second.
147	Core – Pre-Hospital Patient Care	What is a normal pupillary response to light?	Dilation	Constriction (both pupils constrict	No change	One pupil constricts, the other dilates	B	Normal pupils constrict equally and briskly when exposed to light (PEARL: Pupils

				equally)				Equal And Reactive to Light).
148	Core – Pre-Hospital Patient Care	What does PEARL stand for in neurological assessment?	Pulse, Eyes, Airway, Respiration, Level	Pupils Equal And Reactive to Light	Pain, Ears, Airway, Respiration, Lungs	Pressure, Eyes, Arms, Reflexes, Legs	B	PEARL stands for Pupils Equal And Reactive to Light — a quick assessment of neurological function and brain stem integrity.
149	Core – Pre-Hospital Patient Care	What is the Glasgow Coma Scale (GCS) score range?	0-10	3-15	1-20	5-25	B	The GCS ranges from 3 (worst) to 15 (best), assessing eye opening (1-4), verbal response (1-5), and motor response (1-6).
150	Core – Pre-Hospital Patient Care	A GCS score of 8 or below indicates what?	Mild head injury	Severe head injury with potential need for airway protection	No injury	Moderate headache only	B	A GCS of 8 or below indicates severe brain injury; the patient likely cannot protect their airway and may require intubation.
151	Core – Pre-Hospital Patient Care	What are the three components of the Glasgow Coma Scale?	Heart rate, blood pressure, respiration	Eye opening, verbal response, motor response	Pupil size, reflexes, consciousness	Pain, temperature, touch	B	GCS assesses three components: Eye opening (1-4), Verbal response (1-5), and Motor response (1-6), with a total score of 3-15.
152	Core – Pre-Hospital Patient Care	What is the medical term for difficulty breathing?	Dyspnea	Apnea	Bradypnea	Orthopnea	A	Dyspnea is the medical term for difficulty breathing or shortness of breath. Apnea is absence of breathing, bradypnea is slow breathing.
153	Core – Pre-Hospital Patient Care	What is the normal body temperature range for a healthy adult?	35.0-35.5°C	36.1-37.2°C	38.0-39.0°C	40.0-41.0°C	B	Normal body temperature is 36.1-37.2°C (97-99°F). Temperatures above 37.2°C may indicate fever, while below 35°C indicates hypothermia.
154	Core – Pre-Hospital Patient Care	What is the correct site for checking a pulse in an unresponsive adult?	Radial pulse	Carotid pulse	Pedal pulse	Temporal pulse	B	The carotid pulse is the preferred site for checking a pulse in an unresponsive adult because it is central and most reliable during low-perfusion states.
155	Core – Pre-Hospital Patient Care	What does 'tidaling' in a chest drainage system indicate?	System malfunction	Normal function with fluctuation reflecting respiratory cycle	Air leak	Infection	B	Tidaling (fluctuation of fluid level with breathing) in a chest drainage system indicates normal functioning, reflecting pressure changes during respiration.
156	Core – Pre-Hospital Patient Care	What is the correct position for a patient in anaphylactic shock?	Prone position	Supine with legs elevated (if no spinal injury suspected)	Sitting upright	Trendelenburg only	B	A patient in anaphylactic shock should be placed supine with legs elevated to improve venous return, unless spinal injury is suspected.
157	Core – Pre-Hospital Patient Care	What is the first-line medication for anaphylaxis?	Aspirin	Epinephrine	Nitroglycerin	Metoprolol	B	Epinephrine is the first-line treatment for anaphylaxis, reversing vasodilation and bronchoconstriction through alpha and beta-adrenergic effects.
158	Core – Pre-Hospital Patient Care	What is the appropriate dose of epinephrine for anaphylaxis in an adult?	0.01 mg IV	0.3-0.5 mg IM (1:1000)	1 mg IV push	5 mg IM	B	The standard adult dose for anaphylaxis is 0.3-0.5 mg (0.3-0.5 mL of 1:1000 concentration) administered intramuscularly, typically in the lateral thigh.
159	Core – Pre-Hospital Patient Care	What is a key sign of inadequate breathing?	Equal chest rise	Shallow or irregular chest movement	Normal skin color	Clear speech	B	Shallow or irregular chest movement indicates inadequate ventilation and may require assisted ventilation or airway intervention.
160	Core – Pre-Hospital Patient Care	What is the correct method for opening the airway of an unresponsive patient without suspected spinal injury?	Jaw thrust	Head-tilt/chin-lift maneuver	Neck extension only	Finger sweep	B	The head-tilt/chin-lift is the standard method for opening the airway in unresponsive patients without suspected cervical spine injury.
161	Core – Pre-Hospital	What is an oropharyngeal airway (OPA) used for?	To deliver oxygen	To maintain airway	To suction secretions	To measure oxygen	B	An OPA keeps the tongue away from the

	Patient Care			patency in an unresponsive patient by preventing tongue obstruction		saturation		posterior pharynx in unresponsive patients, maintaining airway patency. It should only be used in patients without a gag reflex.
162	Core – Pre-Hospital Patient Care	What is a nasopharyngeal airway (NPA) used for?	Only for nasal bleeding	To maintain airway patency in patients who may have a gag reflex	To deliver nasal medication	To check nasal patency	B	An NPA maintains airway patency and can be used in semi-conscious patients who retain a gag reflex, unlike the OPA which would trigger gagging.
163	Core – Pre-Hospital Patient Care	What is a contraindication for nasopharyngeal airway insertion?	Patient is awake and alert	Suspected basilar skull fracture	Patient has a fever	Patient is over 50 years old	B	Suspected basilar skull fracture is a contraindication because the NPA could pass through a fractured cribriform plate into the cranial vault.
164	Core – Pre-Hospital Patient Care	What is the correct size for an oropharyngeal airway?	Measure from the earlobe to the corner of the mouth	Measure from the corner of the mouth to the angle of the jaw	Measure from the chin to the sternum	Any size will work	B	The correct OPA size is measured from the corner of the mouth to the angle of the jaw (earlobe), ensuring the tip reaches the posterior pharynx.
165	Core – Pre-Hospital Patient Care	What does capillary refill time assess?	Lung function	Peripheral perfusion and circulation status	Kidney function	Brain function	B	Capillary refill time assesses peripheral perfusion — a refill time greater than 2 seconds suggests poor peripheral circulation or dehydration.
166	Core – Transport Patients	Which position is most appropriate for transporting a conscious patient with difficulty breathing?	Supine position	Prone position	Semi-Fowler's position	Trendelenburg position	C	Semi-Fowler's position (head elevated 30-45 degrees) improves breathing by allowing better diaphragm movement and lung expansion.
167	Core – Transport Patients	When transporting a patient with a suspected spinal injury, which device should be used?	Stair chair	Long spine board	Scoop stretcher only	Wheeled stretcher without straps	B	A long spine board provides full spinal immobilization during transport, minimizing the risk of further spinal cord damage.
168	Core – Transport Patients	What is the Trendelenburg position used for?	To treat head injuries	To increase venous return in patients with shock (legs elevated, head down)	To improve breathing	To reduce nausea	B	The Trendelenburg position (feet elevated 15-30 degrees, head down) was traditionally used for shock to increase venous return, though current evidence limits its use.
169	Core – Transport Patients	What is the shock position (modified Trendelenburg)?	Head elevated, feet flat	Supine with legs elevated approximately 12 inches	Prone position	Lateral recumbent position	B	The shock position involves placing the patient supine with legs elevated approximately 12 inches to improve venous return to the heart.
170	Core – Transport Patients	What is the recovery position?	Supine with arms at sides	Lateral recumbent position to maintain airway patency in unresponsive breathing patients	Prone with head to the side	Sitting upright	B	The recovery position (lateral recumbent) keeps the airway open and allows fluids to drain from the mouth in unresponsive but breathing patients.
171	Core – Transport Patients	When should a patient be placed in a left lateral recumbent position during transport?	For all patients	For pregnant patients in the third trimester to prevent supine hypotensive syndrome	For patients with arm fractures	For patients with eye injuries	B	Pregnant patients in the third trimester should be placed left lateral recumbent to prevent the gravid uterus from compressing the inferior vena cava.
172	Core – Transport Patients	What is the purpose of strapping a patient to the stretcher during transport?	To restrain the patient	To secure the patient and prevent falls during transport	To keep the patient warm	To restrict breathing	B	Stretcher straps secure the patient to prevent falls and injuries during ambulance transport, especially during sudden stops or turns.
173	Core – Transport Patients	What is the correct procedure for loading a stretcher into an ambulance?	One person pushes while the other watches	At least two rescuers: one at the head and one at the foot, guiding the stretcher into the locking	The patient loads the stretcher themselves	Only the driver loads the stretcher	B	At least two rescuers are required — one at the head and one at the foot — to safely guide, lift, and secure the stretcher into the ambulance locking mechanism.

				mechanism				
174	Core – Transport Patients	How should a patient with chest pain be positioned during transport?	Prone	Semi-Fowler's or position of comfort	Trendelenburg	Head down	B	Patients with chest pain should be placed in semi-Fowler's or their position of comfort to reduce the work of breathing and cardiac workload.
175	Core – Transport Patients	What is the proper position for a patient with suspected neck injury during transport?	No specific position required	Supine with cervical collar and spinal immobilization	Sitting upright	Side-lying without support	B	A suspected neck injury requires cervical collar application and full spinal immobilization on a long spine board during transport.
176	Core – Transport Patients	What monitoring should be performed during patient transport?	No monitoring is needed during transport	Continuous monitoring of vital signs, mental status, and interventions	Only checking once at the start	Only at the end of transport	B	Continuous monitoring during transport ensures early detection of changes in patient condition and allows timely intervention.
177	Core – Transport Patients	What is the purpose of a transport checklist?	To delay transport	To ensure all equipment, medications, and documentation are prepared before departure	To count the number of passengers	To record fuel levels	B	A transport checklist ensures all necessary equipment, medications, and documentation are prepared, preventing omissions that could compromise patient care.
178	Core – Transport Patients	What is the correct technique for moving a stretcher down stairs?	Feet first with the stretcher tilted slightly toward the rescuers	Head first	Sideways	Carried by one person only	A	When descending stairs, the stretcher should go feet first and be tilted slightly toward the rescuers at the lower end, maintaining control and patient safety.
179	Core – Transport Patients	What is the maximum weight capacity of a standard EMS stretcher?	150 kg	Approximately 225-250 kg (depending on manufacturer)	100 kg	50 kg	B	Standard EMS stretchers typically have a weight capacity of approximately 225-250 kg, though bariatric stretchers can accommodate higher weights.
180	Core – Transport Patients	What is a bariatric stretcher?	A stretcher for children	A stretcher designed to safely transport obese or bariatric patients with higher weight capacity	A stretcher for psychiatric patients	A stretcher for animals	B	A bariatric stretcher is specifically designed with a wider frame and higher weight capacity to safely transport obese patients.
181	Core – Transport Patients	What should be done before moving a patient to a hospital bed?	Just push the stretcher into the room	Communicate with receiving staff, verify patient identity, and provide a verbal handoff report	Leave the patient on the stretcher indefinitely	Transfer without any communication	B	Before transferring, communicate with receiving staff, verify patient identity, and provide a comprehensive verbal handoff report for continuity of care.
182	Core – Transport Patients	What is the purpose of a patient care report during transport?	To keep the provider busy	To document all assessment findings, interventions, patient responses, and communications during the call	To bill the patient only	To record the ambulance mileage	B	The PCR documents the complete record of patient care including assessments, interventions, responses, and communications, serving as a legal and medical document.
183	Core – Transport Patients	What is the function of a stair chair in patient transport?	To transport patients up and down stairs in a seated position	To replace the stretcher completely	To transport patients on rough terrain	To perform physical therapy	A	A stair chair is specifically designed to transport patients up and down stairs in a seated position when a stretcher cannot be used.
184	Core – Transport Patients	When is a non-emergency transport indicated?	When the patient has a life-threatening condition	When a patient needs medical transport without lights and siren (e.g., interfacility transfer)	When no ambulance is available	Never	B	Non-emergency transport is for patients requiring medical monitoring during transport but without an immediate life threat, such as interfacility transfers or discharge transport.
185	Core – Transport Patients	What should be done immediately after completing a patient transport?	Go home immediately	Clean and restock the ambulance, complete documentation, and prepare for the next	Wait for the next shift	Only refuel the ambulance	B	After transport, the ambulance must be cleaned, restocked, and documentation completed to ensure readiness for the

				call				next emergency call.
186	Core – Drive Ambulance	When driving an ambulance under emergency conditions, what is the most important safety consideration?	Arriving as fast as possible	Driving safely and arriving alive	Using the siren continuously	Overtaking all vehicles	B	The paramount rule in emergency driving is to drive safely. Arriving safely is more important than speed, as a crash creates additional casualties.
187	Core – Drive Ambulance	What is the purpose of using lights and siren during emergency response?	To show authority	To alert other drivers and pedestrians to yield the right of way	To clear traffic for fun	To test the equipment	B	Lights and siren are used to request the right of way by alerting other road users to the emergency vehicle's presence and urgency.
188	Core – Drive Ambulance	When approaching an intersection with a red light during emergency response, what should the ambulance driver do?	Proceed through without slowing down	Come to a complete stop, ensure all lanes are clear, then proceed with caution	Speed up to beat the light	Turn off the siren	B	At a red light or stop sign, the ambulance must come to a complete stop, verify all lanes are clear, and proceed only when it is safe to do so.
189	Core – Drive Ambulance	What is 'due regard' in the context of emergency vehicle operation?	Ignoring all traffic laws	Driving with reasonable caution considering the safety of all road users	Driving at maximum speed always	Following the ambulance ahead closely	B	Due regard means operating the emergency vehicle with reasonable care for the safety of all persons, not assuming others will always yield.
190	Core – Drive Ambulance	What should the ambulance driver do when another vehicle does not yield?	Force the vehicle off the road	Slow down or stop and wait for a safe opportunity to pass	Honk continuously and speed up	Call the police on the driver	B	If a vehicle does not yield, the ambulance driver must slow down or stop to avoid a collision, as emergency vehicle operators cannot force others off the road.
191	Core – Drive Ambulance	What is the recommended following distance when driving an ambulance in non-emergency mode?	1 second	3-4 seconds behind the vehicle ahead	10 seconds	Tailgate the vehicle ahead	B	A following distance of 3-4 seconds allows adequate time to react to sudden stops or changes in traffic flow, particularly with the added weight of an ambulance.
192	Core – Drive Ambulance	What is the effect of wet road conditions on ambulance braking distance?	No effect	Braking distance increases significantly	Braking distance decreases	Only affects speed, not braking	B	Wet roads reduce tire traction, significantly increasing braking distance. Ambulance drivers must reduce speed and increase following distance in wet conditions.
193	Core – Drive Ambulance	What should the ambulance driver do before backing up the vehicle?	Just look in the rearview mirror	Use a spotter and walk around the vehicle to check for obstacles	Back up as fast as possible	Honk the horn continuously	B	Before backing up, the driver should use a spotter and walk around the vehicle to identify any obstacles, people, or hazards in the path.
194	Core – Drive Ambulance	What is the purpose of a daily vehicle inspection checklist?	To create paperwork	To identify mechanical issues and ensure the ambulance is safe to operate	To count the number of calls	To record fuel costs only	B	A daily vehicle inspection identifies mechanical issues, fluid levels, tire condition, and equipment functionality, ensuring the ambulance is safe to operate.
195	Core – Drive Ambulance	When should the ambulance driver use emergency lights without siren?	When arriving at a scene to avoid alarming the neighborhood	When parked at a scene for safety and visibility	Never	Only at night	B	Emergency lights without siren are used when the ambulance is parked at a scene to warn approaching traffic and protect the work area.
196	Core – Drive Ambulance	What is the purpose of the ambulance governor (speed limiter)?	To prevent the ambulance from exceeding a safe maximum speed	To increase engine power	To reduce fuel consumption only	To play music in the cabin	A	A governor limits the maximum speed of the ambulance to prevent excessive speed that could lead to loss of control, especially during emergency response.
197	Core – Drive Ambulance	What should the ambulance driver do if the vehicle begins to skid?	Brake hard and steer into the skid	Steer in the direction of the skid and ease off the accelerator	Accelerate to regain traction	Turn off the engine	B	When skidding, steer in the direction the rear of the vehicle is sliding (into the skid) and ease off the accelerator to regain traction without abrupt braking.
198	Core – Drive Ambulance	What is the safest position for the ambulance when parked at a crash scene?	Facing oncoming traffic to block the lane	Angled slightly to protect the work area, with warning lights	Parked on the opposite side of the road	Parked in the middle of the road	B	The ambulance should be angled slightly to create a buffer zone and protect the work area from oncoming traffic, with all

				activated				warning lights activated.
199	Core – Drive Ambulance	What is the meaning of 'code 3' response?	Routine response without lights or siren	Emergency response with lights and siren	Return to station	Cancel the call	B	Code 3 indicates an emergency response requiring the use of lights and siren to request the right of way for urgent patient care.
200	Core – Drive Ambulance	What is the meaning of 'code 2' response?	Emergency response with lights and siren	Urgent response without lights and siren (no life-threatening emergency)	Cancel the call	Mass casualty response	B	Code 2 indicates an urgent but non-life-threatening response that does not require lights and siren, prioritizing safety while maintaining timely arrival.
201	Common – Infection Control	What is the most effective method for preventing the spread of infections in the pre-hospital setting?	Wearing gloves only	Proper hand hygiene	Using antibiotics before each call	Avoiding all patient contact	B	Hand hygiene is the single most effective measure for preventing infection transmission in healthcare and pre-hospital settings.
202	Common – Infection Control	Which type of precautions should be taken for a patient with suspected tuberculosis?	Contact precautions only	Airborne precautions with N95 respirator	No precautions necessary	Droplet precautions with surgical mask	B	Tuberculosis is transmitted via airborne droplet nuclei, requiring airborne precautions including N95 respirator use.
203	Common – Infection Control	What is the minimum time recommended for handwashing with soap and water?	5 seconds	20 seconds	2 minutes	10 minutes	B	The WHO and CDC recommend handwashing for at least 20 seconds with soap and water to effectively remove pathogens.
204	Common – Infection Control	When should alcohol-based hand rub NOT be used?	Before patient contact	When hands are visibly soiled or contaminated with blood/body fluids	After removing gloves	Before eating	B	Alcohol-based hand rub is not effective when hands are visibly soiled; soap and water must be used first to remove organic material.
205	Common – Infection Control	What is Standard Precautions?	Precautions used only for patients with known infections	Infection control practices applied to all patients regardless of suspected infection status	Precautions only for HIV patients	Precautions only used in hospitals	B	Standard Precautions are applied to ALL patients, treating all blood and body fluids as potentially infectious, regardless of known infection status.
206	Common – Infection Control	What PPE is required for contact with blood or body fluids?	Only a mask	Gloves at minimum; add gown, mask, and eye protection as needed based on exposure risk	Only a gown	No PPE is required	B	Gloves are the minimum PPE for blood/body fluid contact. Additional PPE (gown, mask, eye protection) is added based on the risk of exposure.
207	Common – Infection Control	What is the correct sequence for removing PPE?	Gloves, gown, mask, eye protection	Gloves first, then eye protection/gown, then mask/hand hygiene	Mask first, then gloves	Any order is acceptable	B	The correct removal sequence minimizes contamination: remove gloves first (most contaminated), then eye protection/gown, then mask, and perform hand hygiene.
208	Common – Infection Control	What should be done with needles after use?	Recap the needle and dispose in the trash	Dispose immediately in a sharps container without recapping	Leave on the patient's bed	Bend the needle and throw in the regular trash	B	Used needles must be placed directly in a sharps container without recapping to prevent needlestick injuries, a leading cause of occupational exposure.
209	Common – Infection Control	What is a needlestick injury?	A cut from a scalpel	An accidental puncture wound from a used needle, posing infection risk	A burn injury	A muscle strain	B	A needlestick injury is a percutaneous puncture wound caused by a used needle, posing risk of bloodborne pathogen transmission (HIV, Hepatitis B/C).
210	Common – Infection Control	What is the first action after a needlestick injury?	Continue working	Wash the wound with soap and water and report the incident immediately	Apply a bandage and ignore it	Squeeze the wound to remove blood	B	After a needlestick, wash the wound with soap and water, report the incident immediately, and follow post-exposure protocols for bloodborne pathogens.
211	Common – Infection Control	What are the three major bloodborne pathogens of concern in EMS?	Influenza, common cold, and strep throat	Hepatitis B, Hepatitis C, and HIV	Tuberculosis, malaria, and cholera	E. coli, Salmonella, and norovirus	B	The three major bloodborne pathogens of concern are Hepatitis B virus (HBV), Hepatitis C virus (HCV), and Human

								Immunodeficiency Virus (HIV).
212	Common – Infection Control	What is the purpose of an exposure control plan?	To eliminate all infections	To outline procedures for preventing and managing occupational exposure to infectious materials	To punish employees who get infected	To reduce the number of ambulance calls	B	An exposure control plan details procedures for preventing and responding to occupational exposures, including PPE use, post-exposure protocols, and training requirements.
213	Common – Infection Control	What type of mask is required for droplet precautions?	N95 respirator	Surgical mask	Full-face respirator	No mask needed	B	Droplet precautions require a standard surgical mask, as droplets are large and travel short distances (typically <6 feet). N95 is for airborne precautions.
214	Common – Infection Control	How should biohazardous waste be disposed of?	In the regular trash	In designated biohazard bags and containers per local regulations	By flushing down the toilet	By burning in the open	B	Biohazardous waste must be disposed of in properly labeled biohazard bags and containers according to local, national, and institutional regulations.
215	Common – Infection Control	What is the incubation period?	The time from infection to onset of symptoms	The time from treatment to cure	The duration of the illness	The recovery period	A	The incubation period is the interval between initial infection and the onset of symptoms, varying by pathogen.
216	Common – Handle Difficult Behavior	When encountering a patient displaying aggressive behavior, what should the EMS provider do first?	Physically restrain the patient immediately	Ensure personal safety and maintain a safe distance	Shout at the patient to calm down	Ignore the behavior	B	The provider must first ensure personal safety and maintain a safe distance before attempting de-escalation or any patient interaction.
217	Common – Handle Difficult Behavior	What is de-escalation?	Using physical force to control a patient	Verbal and non-verbal techniques to reduce agitation and prevent escalation of behavior	Ignoring the patient	Calling the police immediately	B	De-escalation uses calm communication, active listening, and body language to reduce a patient's agitation and prevent the situation from worsening.
218	Common – Handle Difficult Behavior	Which communication technique is most effective for a hostile patient?	Arguing with the patient	Active listening and empathetic responses	Threatening the patient	Walking away without explanation	B	Active listening and empathy help validate the patient's feelings, reduce defensiveness, and build rapport, which can de-escalate hostile situations.
219	Common – Handle Difficult Behavior	When is physical restraint of a patient justified?	Whenever the provider feels like it	When the patient poses a danger to self or others and less restrictive measures have failed	For all psychiatric patients	Only for elderly patients	B	Physical restraint is justified only when the patient poses an immediate danger to themselves or others and less restrictive interventions have been attempted.
220	Common – Handle Difficult Behavior	What is the appropriate documentation when restraining a patient?	No documentation needed	Document the reason for restraint, type used, time applied, vital signs, and frequent reassessments	Only the patient's name	Only the time of arrival	B	Complete documentation includes the justification, type of restraint, application time, vital signs, and reassessment findings to protect both patient and provider.
221	Common – Handle Difficult Behavior	What is the legal concept of 'implied consent' in EMS?	Patient verbally consents	Consent assumed for unresponsive or incapacitated patients in emergency situations	Consent from a minor	Consent given in writing only	B	Implied consent allows EMS providers to treat unresponsive or incapacitated patients in emergencies, assuming they would consent if able.
222	Common – Handle Difficult Behavior	What is 'expressed consent'?	Consent implied by the situation	Verbal or written permission given by a competent patient	Consent from a family member only	Consent from a police officer	B	Expressed consent is explicitly given by a competent patient, either verbally or in writing, after being informed of the treatment and risks.
223	Common – Handle Difficult Behavior	What should an EMS provider do if a competent adult refuses treatment?	Treat the patient anyway	Respect the refusal, document it, and have the patient sign a refusal form	Call the police to force treatment	Restrain the patient	B	Competent adults have the right to refuse treatment. The provider must inform them of risks, obtain a signed refusal, and document the interaction thoroughly.

224	Common – First Aid	What is the correct first aid for a minor superficial burn?	Apply butter or oil	Cool the burn with running cool water for at least 10 minutes	Apply ice directly	Pop any blisters immediately	B	Cooling with running cool water for at least 10 minutes reduces tissue damage and pain. Butter, oil, and ice can worsen the injury.
225	Common – First Aid	In managing epistaxis (nosebleed), which action is correct?	Tilt the head backward	Pinch the soft part of the nose and lean forward	Lie the patient flat on their back	Insert tissue deep into the nostril	B	Pinching the soft nose while leaning forward applies direct pressure and prevents blood from draining into the throat, which could cause aspiration.
226	Common – First Aid	What is the correct first aid for a choking adult who is conscious but cannot speak?	Give water	Perform abdominal thrusts (Heimlich maneuver)	Slap the back while the person is lying down	Wait for the object to pass	B	Abdominal thrusts (Heimlich maneuver) create pressure to dislodge the airway obstruction in a conscious choking adult who cannot speak or cough effectively.
227	Common – First Aid	What is the correct first aid for a suspected fracture?	Attempt to realign the bone	Immobilize the injured area and seek medical attention	Apply a tight bandage above the fracture	Massage the area	B	Suspected fractures should be immobilized to prevent further injury, reduce pain, and minimize the risk of damaging surrounding tissues and blood vessels.
228	Common – First Aid	What is the correct management for a snake bite?	Apply a tourniquet and cut the wound	Keep the patient calm and still, immobilize the bitten limb, and transport to hospital	Suck out the venom	Apply ice directly to the bite	B	Keep the patient calm and still to slow venom absorption, immobilize the limb at heart level, and transport immediately. Tourniquets, cutting, and suction are contraindicated.
229	Common – First Aid	What is the RICE method for treating sprains?	Rest, Ice, Compression, Elevation	Rest, Immobilize, Cover, Examine	Run, Ice, Carry, Elevate	Rest, Inspect, Compress, Extend	A	RICE stands for Rest, Ice, Compression, and Elevation — the standard first aid treatment for soft tissue injuries like sprains and strains.
230	Common – First Aid	What is the correct management for an impaled object in a wound?	Remove the object immediately	Stabilize the object in place and transport	Push the object through	Cut the object short	B	Impaled objects should be stabilized in place to prevent further damage and control bleeding. Removal may cause uncontrolled hemorrhage.
231	Common – First Aid	What is the correct first aid for a seizure?	Restrain the patient and put something in their mouth	Protect the patient from injury, do not restrain, and position on their side after the seizure	Hold the patient down firmly	Pour water on the patient	B	During a seizure, protect the patient from injury without restraining. After the seizure, place them in the recovery position. Never put objects in the mouth.
232	Common – First Aid	What is the correct first aid for a dog bite?	Apply a tourniquet	Wash the wound with soap and water, control bleeding, and seek medical attention	Ignore the bite	Apply butter to the wound	B	Dog bites should be washed with soap and water, bleeding controlled with direct pressure, and medical attention sought for assessment of rabies risk and wound management.
233	Common – First Aid	What is the correct management for a chemical burn?	Apply a bandage immediately	Flush with copious amounts of water for at least 20 minutes	Apply butter	Rub the area	B	Chemical burns require immediate and prolonged flushing with copious water to dilute and remove the chemical, preventing ongoing tissue damage.
234	Common – First Aid	What is the correct first aid for an insect sting?	Squeeze the sting site	Remove the stinger by scraping, clean the area, apply cold compress	Apply heat	Ignore it	B	Scrape the stinger away (not squeeze), clean the area, and apply a cold compress to reduce swelling and pain. Watch for signs of anaphylaxis.
235	Common – First Aid	What are the signs of a closed fracture?	Bone protruding through the skin	Swelling, deformity, pain, and crepitus without an open wound	No pain or swelling	Bleeding only	B	A closed fracture presents with swelling, deformity, pain, tenderness, and possibly crepitus, but the skin remains intact without an open wound.
236	Common – First Aid	What is an open fracture?	A fracture with no skin break	A fracture where the bone has broken through the skin or a	A dislocated joint	A muscle tear	B	An open (compound) fracture involves a bone fragment breaking through the skin or a wound communicating with the

				wound communicates with the fracture				fracture site, increasing infection risk.
237	Common – Patient Service Standards	What is the primary goal of maintaining high patient service standards?	To receive awards	To ensure quality patient care and patient satisfaction	To complete paperwork	To compete with other agencies	B	High patient service standards ensure quality patient care, leading to better outcomes and patient satisfaction — the central goal of EMS.
238	Common – Patient Service Standards	What is patient confidentiality?	Sharing patient information freely	Protecting patient information from unauthorized disclosure	Discussing patients on social media	Telling family members everything	B	Patient confidentiality means protecting personal health information from unauthorized access or disclosure, a legal and ethical obligation in healthcare.
239	Common – Patient Service Standards	What is informed consent?	Consent obtained under pressure	Permission given after the patient understands the treatment, risks, benefits, and alternatives	Consent from a minor only	Consent without any explanation	B	Informed consent requires that the patient understands the proposed treatment, its risks and benefits, and alternatives before giving permission.
240	Common – Patient Service Standards	What is the patient's bill of rights?	A list of rules patients must follow	A set of guarantees protecting patients' rights to respectful, informed, and confidential care	A billing statement	A list of hospital services	B	The patient's bill of rights guarantees protections including the right to informed consent, confidentiality, respectful treatment, and refusal of care.
241	Basic – Workplace Communication	Which best describes effective workplace communication in EMS?	Giving orders without listening	Gathering, conveying, and storing information accurately and promptly	Communicating only during emergencies	Sharing personal opinions during handover	B	Effective workplace communication involves the accurate and timely gathering, conveying, and storing of information for coordinated patient care.
242	Basic – Workplace Communication	What is the purpose of a shift change briefing?	To socialize with colleagues	To transfer critical information about ongoing operations, equipment status, and pending calls	To count inventory	To assign parking spaces	B	Shift change briefings transfer essential operational information, equipment status, and pending calls to ensure continuity of service and safety.
243	Basic – Workplace Communication	What is active listening?	Hearing words without paying attention	Fully concentrating on the speaker, understanding the message, and providing feedback	Interrupting the speaker frequently	Thinking about your response while the other person speaks	B	Active listening involves fully concentrating on the speaker, understanding the message, and providing verbal and non-verbal feedback to confirm understanding.
244	Basic – Workplace Communication	Why is accurate documentation important in EMS?	To fill time during shifts	To provide a legal record, ensure continuity of care, and support quality improvement	Only for billing purposes	To count the number of calls	B	Accurate documentation provides a legal record of care, supports continuity between providers, and enables quality improvement analysis.
245	Basic – Workplace Communication	What is the purpose of a written incident report?	To assign blame	To document facts about an unusual occurrence for legal and quality review purposes	To create publicity	To count the number of patients	B	Incident reports document factual details of unusual occurrences (accidents, errors, complaints) for legal protection, quality review, and process improvement.
246	Basic – Teamwork	What is the key benefit of effective teamwork in EMS?	Individual recognition	Improved patient outcomes through coordinated and efficient care delivery	Reduced need for training	Elimination of supervision	B	Effective teamwork ensures coordinated, efficient care delivery, reduces errors, and leads to improved patient outcomes — the central goal of EMS.
247	Basic – Teamwork	What is a 'crew resource management' (CRM) concept in EMS?	Managing the ambulance fleet	Using all available resources (people, equipment, information) effectively for safe and efficient operations	Managing patient resources only	Assigning tasks based on seniority only	B	CRM utilizes all available resources — personnel, equipment, and information — to enhance safety, communication, and decision-making in EMS operations.

248	Basic – Teamwork	What is the role of a team leader during an emergency call?	To do all tasks alone	To direct and coordinate team activities while ensuring patient care standards are met	To delegate all tasks and observe only	To drive the ambulance only	B	The team leader directs and coordinates activities, assigns roles, ensures standards are maintained, and oversees the overall patient care during the call.
249	Basic – Teamwork	What is constructive feedback?	Criticizing a colleague publicly	Specific, actionable information given to improve performance in a respectful manner	Ignoring mistakes	Only positive comments	B	Constructive feedback provides specific, actionable information about performance in a respectful manner, aimed at improvement rather than blame.
250	Basic – Teamwork	What is the purpose of a post-call debriefing?	To gossip about the patient	To review the call, discuss what went well and what could be improved	To assign blame for errors	To plan social activities	B	Post-call debriefing is a structured review of the call to identify what went well, areas for improvement, and emotional processing for the team.
251	Basic – Professionalism	Which action best demonstrates career professionalism in EMS?	Arriving late for shifts	Setting work priorities and maintaining competence through continuous learning	Refusing tasks outside one's comfort zone	Gossiping about colleagues	B	Professionalism involves setting priorities, maintaining competence through continuous education, and demonstrating accountability and integrity.
252	Basic – Professionalism	What is the purpose of continuing education in EMS?	To earn more money only	To maintain and update knowledge and skills in accordance with evolving standards and protocols	To avoid working shifts	To become a doctor	B	Continuing education ensures EMS providers maintain current knowledge and skills, adapt to protocol changes, and deliver evidence-based patient care.
253	Basic – Professionalism	What is ethical behavior in EMS?	Treating patients differently based on social status	Adhering to moral principles including honesty, integrity, confidentiality, and equitable patient care	Accepting gifts from patients in exchange for preferential treatment	Sharing patient information freely	B	Ethical behavior in EMS requires honesty, integrity, confidentiality, and providing equitable care to all patients regardless of background or status.
254	Basic – Professionalism	What is the purpose of a code of ethics in EMS?	To create rules for punishment	To provide a framework of professional standards and moral principles guiding conduct	To increase paperwork	To reduce the number of employees	B	A code of ethics establishes professional standards and moral principles that guide EMS providers' conduct and decision-making in patient care.
255	Basic – Professionalism	What is stress management important for in EMS?	Only for personal comfort	Preventing burnout, maintaining mental health, and ensuring safe and effective patient care	To avoid work	To get more vacation time	B	Stress management prevents burnout, protects mental health, and ensures providers can deliver safe and effective patient care over their career.
256	Basic – Occupational Health & Safety	What is the primary purpose of following occupational health and safety procedures?	To avoid legal liability only	To protect the health and safety of EMS providers and others in the workplace	To increase workload	To satisfy patient demands	B	OHS procedures protect EMS providers and others from workplace hazards, ensuring a safe environment and reducing the risk of injury or illness.
257	Basic – Occupational Health & Safety	What is the purpose of a risk assessment in the workplace?	To create more paperwork	To identify hazards, evaluate risks, and implement control measures to prevent injuries	To count the number of employees	To determine salary levels	B	Risk assessment identifies workplace hazards, evaluates the likelihood and severity of harm, and implements controls to eliminate or reduce risks.
258	Basic – Occupational Health & Safety	What is the hierarchy of controls for managing workplace hazards?	PPE first, then elimination	Elimination, substitution, engineering controls, administrative controls, PPE (last resort)	Administrative controls only	PPE only	B	The hierarchy prioritizes elimination (most effective) through substitution, engineering, and administrative controls, with PPE as the last line of defense.
259	Basic – Occupational	What is the purpose of Material Safety Data Sheets	To list product prices	To provide	To advertise products	To record employee	B	MSDS/SDS provides critical information

	Health & Safety	(MSDS)?		information on hazardous chemicals including safe handling, storage, and emergency procedures		attendance		on hazardous chemicals including properties, safe handling procedures, first aid measures, and emergency protocols.
260	Basic – Occupational Health & Safety	What is the correct action if an EMS provider identifies a new workplace hazard?	Ignore it	Report it immediately to the supervisor and implement temporary controls if safe to do so	Wait for the next meeting to discuss it	Only report if someone gets hurt	B	Newly identified hazards must be reported immediately and temporary controls implemented to protect workers while permanent solutions are developed.